

Leukemia Multidisciplinary Tumor Board Summary

DATE OF CONFERENCE: 15-Nov-2022

PATIENT: CARTER, Ronald

MRN: AML035

DOB: 03-Nov-1949

CASE PRESENTED BY: Dr. Vance

HISTORY:

73 y/o M dx'd w/ AML 01-Sep-2021. PMHx: CAD s/p 2-vessel CABG (2015), HLD, HTN.

- **Diagnostic BMBx:** 65% blasts. Cytogenetics normal.
- **Molecular:** FLT3-ITD positive (high allelic ratio), EZH2 co-mutation. Classifies as **ELN 2022 Adverse-risk**.

TREATMENT COURSE:

- **1L Therapy (started 10-Sep-2021):** Given FLT3+ status and age, initiated targeted therapy with single-agent **Gilteritinib** 120 mg daily.
- **Response:** Achieved a CRi (CR w/ incomplete count recovery) after 3 cycles. Maintained this response for approximately 10 months. Tolerated therapy w/ Grade 2 diarrhea and mild QTc prolongation managed with dose reduction to 80 mg.
- **Relapse (Oct-2022):** Presented with increasing fatigue. PB smear showed circulating blasts. BMBx confirmed molecular relapse (FLT3-ITD still present) and frank morphologic relapse (50% blasts). Disease is now refractory to Gilteritinib after a 13-month response duration.

PATHOLOGY REVIEW (Dr. Singh):

Slides from recent BMBx reviewed. Confirms relapse of AML with myelomonocytic features. No new clonal cytogenetic evolution noted.

DISCUSSION / QUESTIONS FOR THE BOARD:

Case presented for discussion of viable salvage therapy options. Pt's performance status has declined (ECOG 3). He is not a candidate for intensive salvage chemo (e.g., FLAG-Ida) or allogeneic SCT. Options considered:

1. **Venetoclax-based combos (e.g., VEN-HMA):** Standard salvage option.
2. **Clinical Trial:** Screened for local trials. No slots available for FLT3-refractory AML.
3. **Hospice/Palliative Care:** Given adverse genetics (EZH2 + FLT3), short duration of 1L response, and poor PS.

CONSENSUS RECOMMENDATION:

The board agrees that the likelihood of a meaningful or durable response to further cytotoxic therapy is exceedingly low and carries a high risk of toxicity. The consensus is that the most appropriate path forward is a goals-of-care discussion centered on transitioning to palliative care. Further anti-leukemic therapy is not recommended. Dr. Vance will discuss these recommendations with the patient and his family.

EPILOGUE (Added to chart record 05-May-2023):

Patient Carter opted for hospice care following the tumor board discussion. He was managed at home for several months with a focus on symptom control. He passed away from complications of his leukemia on **May 2, 2023**.